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Understanding Sacroiliac (SI) Joint Pain and Treatment Options: Patient Information Leaflet

This leaflet provides information about a common cause of lower back and buttock pain called Sacroiliac (SI) Joint Dysfunction or Mediated Pain. It explains what the SI joint is, the symptoms it causes, and the range of treatment options available. Please use this information to help you in your discussions with your doctor.

What is the Sacroiliac (SI) Joint?

Your pelvis is made up of three bones: the sacrum (the triangular bone at the base of your spine) and the two ilium bones, one on each side. The Sacroiliac (SI) joints are the two large, "L"-shaped joints that connect your spine to your pelvis.

- They are strong, weight-bearing joints that transfer the force from your upper body to your legs.
- They have very little movement, but what little motion they do have is critical for absorbing shock and maintaining stability when you walk, twist, or stand up.
- Like other joints, they have cartilage and can be a source of pain.

What is SI Joint Mediated Pain?

SI Joint Mediated Pain occurs when one or both of these joints become inflamed (sacroiliitis) or dysfunctional. This can happen for several reasons:

- Trauma: A sudden impact like a fall, car accident, or a misstep.
- Repetitive Stress: From activities like jogging, heavy lifting, or uneven leg length.
- Arthritis: Osteoarthritis (wear-and-tear) or inflammatory arthritis (like Ankylosing Spondylitis).
- Pregnancy & Childbirth: Hormones loosen the ligaments, and the added stress can irritate the joint.
- Previous Spine Surgery: Fusion surgery in the lower spine can place increased stress on the SI joints.

Common Symptoms

Pain from the SI joint is often mistaken for a disc problem or other back issues. Key symptoms include:

- A sharp, stabbing, or dull ache low in the back, below the belt line, and deep within the buttock.
- Pain is can be on one side or both sides.
- Pain can radiate to the groin, the back of the thigh, and occasionally down the calf (but rarely into the foot). It does not follow a specific nerve root pattern like sciatica.
- Pain is often worse with;
 - Sitting for long periods (especially on hard surfaces).
 - Standing up from a seated position.
 - Climbing stairs.
 - Running or taking long strides.
- A feeling of stiffness or instability in the pelvis and lower back.
- The joint may be tender to the touch.

Treatment Options

Treatment follows a step-by-step approach, starting with conservative management.

Step 1: Conservative (Non-Invasive) Treatments

These are the first and most important steps in managing SI joint pain.

- **Physiotherapy:** Focuses on core strengthening, hip stabilization, and correcting muscle imbalances and gait.
- **Manual Therapy:** A chiropractor or osteopath may perform specific manipulations to improve joint mobility.
- **Medication:** Anti-inflammatory drugs (like ibuprofen) or pain relievers (like paracetamol).
- **SI Joint Belt:** A specialized belt worn around the hips can provide compression and stability, reducing pain.
- **Activity Modification:** Avoiding movements that provoke the pain.

If these treatments do not provide sufficient relief, your doctor may recommend interventional procedures.

Step 2: Therapeutic Cortisone Injection

If SI I joint is suspected to be a source of pain a cortisone injection can provide relief.

What is it?

A mixture of a local anaesthetic (numbing medicine) and a corticosteroid (a powerful anti-inflammatory) is injected directly into the SI joint structures.

How is it done?

The procedure is performed using X-ray or ultrasound guidance to ensure the needle is placed accurately inside the joint.

Outcome

The steroid reduces inflammation, providing pain relief that can last from several weeks to many months. In some cases steroids do not give meaningful pain relief.

Step 3: Sacral Lateral Branch Blocks leading to Radiofrequency Ablation (RFA)

If you get good but temporary relief from cortisone injection, a longer-lasting treatment may be an option.

Part A: Diagnostic Sacral Lateral Branch Block

The pain from the SI joint is carried by a network of small nerves called the sacral lateral branches.

- In this test, a local anaesthetic is injected onto these specific nerves to block the pain.
- This is a crucial test for RFA. If your pain is reduced by more than 75-80% for the duration of the anaesthetic, it predicts that RFA will be successful.

Part B: Radiofrequency Ablation (RFA) / Neurotomy

If the diagnostic block is successful, RFA can be performed.

What is it?

A procedure that uses heat energy to create a precise lesion on the sacral lateral branch nerves, preventing them from transmitting pain signals to your brain.

What to expect:

The procedure is done under X-ray guidance. Pain relief is not immediate and may take 2-6 weeks to fully develop. The relief can last for 12 months or longer. The nerves can regenerate, so the procedure can be repeated if necessary. The procedure can be done with or without sedation.

Step 4: SI Joint Fusion Surgery

Surgery is considered only after all other non-surgical and less invasive treatments have failed to provide adequate, long-term relief.

What is it?

SI Joint Fusion is a minimally invasive procedure designed to permanently stabilize the painful joint.

How does it work?

The surgeon places small, triangular implants and/or screws across the SI joint through a small incision. This "fuses" the sacrum to the ilium, stopping the painful motion.

When is it an option?

- You have a confirmed SI joint diagnosis (via diagnostic injections).
- You have severe, chronic pain that significantly impacts your quality of life.
- You have not responded to extensive conservative care and injections.

Recovery: This is a significant procedure requiring a period of restricted weight-bearing and several months of physiotherapy for full recovery.

Summary and Key Points

SI Joint pain is a distinct and common cause of low back and buttock pain.

- A diagnostic nerve block injection is the primary tool to confirm the joint as the pain source.
- Therapeutic injections with cortisone can provide medium-term relief.
- For longer-term relief, diagnostic sacral lateral branch blocks are used to determine if you are a good candidate for Radiofrequency Ablation (RFA).
- SI Joint Fusion surgery is a last-resort option for carefully selected patients with confirmed SI joint pain that has not responded to all other treatments.

Your treatment path should be a shared decision between you and your pain specialist or surgeon. They will discuss the potential benefits, risks, and expected outcomes of each option in detail.

Questions? Contact Your Care Team

Before/after procedure:

 Pain Clinic: 782-377-7813

 Info@redsandspm.com

In an emergency

→ Call 911, or visit your local emergency department

Disclaimer: This leaflet is for informational purposes only and is not a substitute for professional medical advice, diagnosis, or treatment. Always seek the advice of your physician or other qualified health provider with any questions you may have.